

Artifact 2a — Home Care Nurse (HCN)

Consolidated Interview Summary

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PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2a	Persona Group	Home Care Nurse (HCN)
Next Step	Feed into competitive-gap-analysis alongside Artifacts 2b, 2c, 2d		

DOCUMENT DETAILS

Source Interviews	HCN-001 (Maria Santos), HCN-002 (David Nguyen), HCN-003 (Priya Sharma)	Synthesis Date	2026-03-27
Interviewer	PM Lead		

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Consolidated Interview Brief

Dates	HCN-001: 2026-03-25 (42 min) HCN-002: 2026-03-26 (38 min) HCN-003: 2026-03-27 (48 min)
Participants	- Maria Santos — Home Care Nurse, CareBridge, eastern suburbs. 14 patients/week. Senior, experienced. - David Nguyen — Home Care Nurse, CareBridge. 10 patients/week. Junior (2 years post-graduation). First year writing care plans. - Priya Sharma — Home Care Nurse (Team Lead), CareBridge, eastern region. 8 own patients + clinical oversight for 5 nurses / ~50 patients total. 18+ years clinical experience including coroner's inquest testimony.
Background	All three are community-based registered nurses delivering home care in the eastern suburbs. They work the same agency EMR, see patients alone, and are responsible for clinical documentation, medication management, and escalation decisions — without in-person peer support. Maria represents the high-volume experienced practitioner. David represents the junior nurse navigating solo decision-making for the first time. Priya represents the team lead who holds systemic risk visibility but is structurally overloaded. Despite their different seniority levels, all three described the same core loop: visit → observe → document → hope the system catches what they missed.

Current Solution

All three use a shared toolkit:

Agency EMR	the primary record system. Described by all three as slow, crash-prone on mobile, lacking trend views or validation logic.
WhatsApp	used for urgent family and coordinator communication (Maria, all).
Personal workarounds	Maria: voice recorder + paper wound notebook. David: phone abbreviations expanded later. Priya: personal spreadsheet (3–4 hrs/Sunday) + personal notebook (every visit, timestamped, for legal protection).
Memory	all three cited memory as the primary carrier of patient context that no system captures.

What They Like About Current Solution

Job to Be Done	What Works	Importance	Satisfaction
Communicate urgent updates	WhatsApp is fast and familiar	High	Medium — informal, no audit trail
Document wound care	Paper notebook allows photos and freeform notes	Medium	Low — duplicates into EMR required
Legal/compliance documentation	Priya's personal notebook catches granular detail	High (for Priya)	Low — doubles documentation time
Care plan peer review	Priya's Sunday review catches ~60% of errors	High	Low — 3-day delay, covers 60% not 100%

Problems With Current Solution

Problem 1 — Pattern Blindness at Scale

Job:	Identify patient deterioration trends before a crisis visit.
Desired outcome:	See longitudinal data (BP, mobility, medication adherence) across multiple visits in one view.
Importance:	Critical — missed patterns lead to preventable hospitalisations.
Satisfaction:	Very low — zero trend view in the EMR; each visit note is an island.

*"I knew something was off with Arthur. I could feel it. But when you're seeing fourteen people a week, you can't hold all the patterns in your head. **The system should have caught that, not me.**"*

— Maria, HCN-001

*"Margaret's hospitalisation was preventable. Every single data point was in the system. **But nobody connected them because nobody has time to read eight weeks of visit notes side by side.**"*

— Priya, HCN-003

Synthesis: Both Maria and Priya experienced preventable adverse events directly caused by pattern blindness. The data existed. The signal was absent. This is the highest-severity pain in the HCN persona — it has direct patient safety consequences and generates lasting professional guilt. David has not yet experienced a patient-harm event from this cause, but his escalation uncertainty (Mrs. Patterson's bruise, 3-day review wait) shows the same structural gap manifesting at junior level.

Problem 2 — Documentation Burden Consuming Clinical Time

Job:	Accurately document every patient visit.
Desired outcome:	Capture visit details in real time, in the right structured fields, without needing to return to a desk.
Importance:	High — both clinical quality and legal compliance depend on documentation.
Satisfaction:	Very low — documentation happens hours after visits, from reconstructed memory.

"I spend more time writing about care than delivering it. By the time I sit down to type up today's visits, I've already forgotten half the details. I'm reconstructing, not documenting."

— Maria, HCN-001

"I'm typing notes on a phone screen in someone's kitchen while their cat sits on my lap. Of course I make mistakes."

— David, HCN-002

"The solicitor asked me what time I gave Gerald his insulin. I said about 8:15. He said, 'Show me.' All my notes said was 'morning visit.'"

— Priya, HCN-003

Synthesis: Documentation failure has two distinct flavours. For Maria, it is a time and fidelity problem — she reconstructs from memory at night and loses detail. For David, it is an accuracy and safety problem — mobile entry in noisy environments causes errors (mg vs mL) that have near-miss consequences. For Priya, it is a legal protection

problem — a coroner's inquest established that vague documentation is professionally indefensible regardless of whether the care was actually delivered. All three converge on the same structural cause: the documentation system was designed for a desk, not a patient's home.

Problem 3 — Blank-Page Care Plans with No Clinical Decision Support

Job:	Write care plans that are clinically appropriate for each patient's specific condition profile.
Desired outcome:	A starting-point recommendation based on assessment data, adjustable by clinical judgment.
Importance:	High — 30% of care plans reviewed by Priya require material corrections.
Satisfaction:	Very low — blank template, no suggestions, no contraindication checks.

"Every care plan I write starts from scratch. No starting point, no suggestion, nothing. Just me and a blank page."

— Maria, HCN-001

"I stared at that blank care plan template for twenty minutes before I started typing. I knew what Dorothy needed in general, but the specifics — I just didn't have the confidence."

— David, HCN-002

"If the system said: 'Based on this assessment, here are recommended goals and interventions, adjusted for this patient's medication contraindications' — I'd spend my time refining good drafts instead of rewriting bad ones from scratch."

— Priya, HCN-003

Synthesis: The blank care plan is a consistent failure across all three HCN interviews. For Maria it means slow, effortful creation. For David it means 90-minute first attempts, submitted with low confidence, returned with six corrections. For Priya it means a 30% correction rate on junior plans, consuming 4–5 hours/week of her oversight time. The downstream impact: junior nurses develop slower, teams spend senior time on rework, and patient plans sit on incorrect goals until Priya's Sunday review cycle catches them.

Problem 4 — Medication Reconciliation Failures and Lack of Input Validation

Job:	Ensure every patient is on the correct dose of every medication, with changes reconciled across all providers.
Desired outcome:	Proactive alerts when medication changes occur; input validation at point of entry.
Importance:	Critical — errors cause preventable harm (Arthur's hospitalisation, David's near-miss, Gerald's death).
Satisfaction:	Very low — changes arrive late, documentation has no validation, reconciliation is manual.

"I'm supposed to be the one coordinating, but I'm the last person in the chain to know when something changes. I spend thirty minutes per patient per month just reconciling medication lists. That's seven hours a month on medication admin."

— Maria, HCN-001

"The mg/mL thing still keeps me up at night. If the system had flagged it — 'this medication is usually measured in mL, did you mean mg?' — I would have caught it instantly."

— David, HCN-002

*"Medication reconciliation failures are probably our highest-frequency clinical risk. **Changes happen at hospital discharge, at GP visits, at specialist appointments. By the time the information reaches us, the patient has already been on the wrong dose for days.**"*

— Priya, HCN-003

Synthesis: Medication failures appear in all three interviews and across the full severity spectrum — near-misses (David's mg/mL error), preventable deterioration (Arthur's missed potassium adjustment), and death (Gerald, Priya's coroner's inquest). The failure modes are structural: no cross-provider change notifications, no input validation in the EMR, and a reconciliation process that is entirely manual and always delayed.

Problem 5 — Clinical Loneliness and Unstructured Escalation

Job:	Make sound escalation decisions about ambiguous clinical situations without peer support.
Desired outcome:	Access to prior visit context + a confidence signal before deciding whether to escalate.
Importance:	High — under-escalation risks patient harm; over-escalation erodes GP trust.
Satisfaction:	Very low — no real-time triage support, no priority differentiation in the review queue.

*"In the hospital, I'd ask a senior. Here, I'm on my own, standing in someone's lounge room trying to make a clinical judgment with **no second opinion available.**"*

— David, HCN-002

*"**Loneliness. Mine, I mean. Not the patients.** Being a home care nurse is lonely. You're making clinical decisions by yourself all day. There's no one to ask 'Does this look right to you?'"*

— David, HCN-002

*"The system puts everything in the same queue with the same priority. A missed medication and a potential fall **look identical in the interface.**"*

— Priya, HCN-003

Synthesis: This pain point is most acute for David — it surfaced unprompted and was described with visible emotion (hands shaking). Maria compensates with experience and intuition. Priya compensates with a Sunday review process. But the structural problem is the same: home care nursing removes the ward team that would normally provide real-time clinical consultation. The EMR's flat, undifferentiated review queue compounds this by failing to signal which flags are urgent.

Key Insights

Insight 1 — The 60% Problem (Priya, HCN-003)

*"I estimate I catch sixty percent of the meaningful trends in my Sunday reviews. **Sixty percent is not good enough. Margaret was in the forty percent.**"*

— Priya, HCN-003

This is the most precise quantification of risk we have from any HCN interview. Priya — arguably the most diligent nurse in the cohort — accepts a 40% miss rate as an unavoidable consequence of a system that requires manual trend detection. Margaret's preventable hospitalisation was not a failure of Priya's skill. It was a failure of the system's inability to surface pattern information she would have acted on.

Insight 2 — Audit Trauma as Documentation Motivation (Priya, HCN-003)

*"The solicitor asked me what time I gave Gerald his insulin. I said about 8:15. He said, 'Show me.' All my notes said was 'morning visit.' **I was telling the truth. I provided competent care. But I couldn't prove it.**"*

— Priya, HCN-003

Priya's coroner's inquest is a rare, high-fidelity signal: legal defensibility is not a future concern for this persona — it is a lived experience. She now double-documents every visit (notebook + EMR) as a direct result. This behaviour is a strong design signal: timestamped, field-specific, immutable documentation has inherent value to nurses beyond just clinical quality. It is professional self-protection. David does not yet have this motivation — but he may, after his mg/mL near-miss.

Insight 3 — The Data Parent Problem (Maria, HCN-001)

*"Sarah was sending me screenshots of her mother's step count and sleep data from a wristband app. Every week: 'Her steps are down forty percent, is something wrong?' **Each one added ten to fifteen minutes to my day** because I'd have to look at the data, compare it with what I observed, and then respond carefully so I didn't escalate Sarah's anxiety."*

— Maria, HCN-001

Raw patient data sent directly to family members without clinical interpretation creates professional friction for nurses. This is a systems design warning: an alert system that notifies families before clinicians review the signal will increase nursing workload, not decrease it. Any notification architecture must route through clinical triage first.

Insight 4 — "I'm Reconstructing, Not Documenting" (Maria, HCN-001)

*"By the time I sit down to type up today's visits, I've already forgotten half the details. I'm **reconstructing, not documenting.**"*

— Maria, HCN-001

This single sentence defines the documentation quality failure. Notes written hours after visits from reconstructed memory are clinically degraded. This is not a nurse behaviour problem — it is a system design problem. The EMR is unusable in the field; the result is deferred documentation that is structurally less accurate.

Insight 5 — "Let Me Just Talk" (David, HCN-002)

*"Let me just talk. I'm standing in someone's kitchen, I've just done a wound assessment. Let me say what happened and have the system figure out where it goes. **Not me typing on a phone screen with autocorrect fighting me.**"*

— David, HCN-002

Voice-to-structured-record is not just a convenience feature for David — it is the only documentation modality that is physically compatible with the field context. This is the clearest direct feature signal in the HCN cohort.

Insight 6 — Pattern Recognition as the Nurse's Undelagatable Job (Priya, HCN-003)

"I don't need a system that replaces clinical judgment. I need a system that does the pattern recognition I can't do at scale, and then puts the pattern in front of me so I can make the judgment call."

— Priya, HCN-003

This is the sharpest articulation of the human-AI division of labour from any interview in the study. It directly defines the product's appropriate autonomy level: Level 1 (surface the pattern) and Level 2 (present it for human decision). Level 3 (act without the nurse) is explicitly not wanted.

Cross-Interview Pattern Summary

Pain Theme	Maria (HCN-001)	David (HCN-002)	Priya (HCN-003)	Shared?
Pattern blindness / no trend view	Arthur's hospitalisation	Mrs. Patterson bruise (3-day wait)	Margaret's hospitalisation	■ All three
Documentation burden / delayed notes	2–3 hrs nightly reconstruction	Phone errors, 10–12 hrs/week	Double-documents every visit	■ All three
Blank care plan / no decision support	30–45 min/plan, blank start	90 min first plan, 6 corrections	30% of junior plans need rework	■ All three
Medication reconciliation failures	Last to know about dose changes	mg/mL near-miss	Coroner's inquest / Gerald	■ All three
Clinical loneliness	Compensated by experience	Named explicitly, visibly distressed	Compensated by Sunday reviews	■ All three
Legal / audit risk	Implicit	Near-miss incident report	Explicit — coroner's court	Escalates with seniority

What They Need (Verbatim Desired Outcomes)

"I'd kill for something that just told me: 'Here's what's changed since your last visit. Here's what to watch for today.' Five sentences. That's all I need before I walk through the door."

— Maria, HCN-001

"Let me just talk. Let me say what happened and have the system figure out where it goes."

— David, HCN-002

"I don't need a system that replaces clinical judgment. I need a system that does the pattern recognition I can't do at scale, and then puts the pattern in front of me so I can make the judgment call."

— Priya, HCN-003

Opportunity Signals (Pre-OST)

These are not yet solutions — they are problem framings that point toward opportunity areas:

- **Pre-visit briefing** — longitudinal trend digest delivered before each visit ("what's changed since you were last here")
- **Voice-to-structured-record** — field-compatible documentation that works standing in a patient's kitchen
- **Care plan recommendation engine** — assessment-driven starting point that nurses refine, not create from blank
- **Medication change notification** — cross-provider alert when a dose, drug, or interaction changes
- **Prioritised escalation queue** — flags ranked by clinical risk, not arrival order
- **Timestamped, immutable audit log** — legal-defensible record of every clinical action

Action Items

Date	Owner	Action
2026-03-28	PM Lead	Run competitive-gap-analysis with HCN, FC, CC, SR pain signals
2026-03-28	PM Lead	Feed Artifact 2a into market-segmentation-deep-dive alongside other persona summaries
2026-03-28	PM Lead	Investigate voice-to-structured-record feasibility (field context: kitchen, phone, ambient noise)
2026-03-28	PM Lead	Quantify: documentation hours/week across HCN cohort at agency level
2026-03-28	PM Lead	Quantify: care plan correction rate — is 30% (Priya's estimate) consistent across agencies?
2026-03-28	PM Lead	Investigate audit log requirements for legal defensibility (coroner's inquest standard)
2026-03-28	PM Lead	Flag: notification routing rule — family alerts must not bypass clinical triage

Handshake note: This artifact feeds directly into competitive-gap-analysis (HS-DISC-01 predecessor) and opportunity-solution-tree. The verbatim desired outcomes — especially Priya's pattern recognition framing — should anchor the OST's Desired Outcome node for the HCN persona.

